KESTREL HEALTH

SERIES A, NOVEMBER 2026

Kestrel Health

Fast physiotherapy for employees, booked into independent clinics' empty weekday slots

£1.9M

GMV, Q3 2026, finance figure

DOCS/BOARD-UPDATE-2026-Q3.MD

41

employers and 312 clinics active, end of September 2026

DATA/KPIS-WEEKLY.CSV

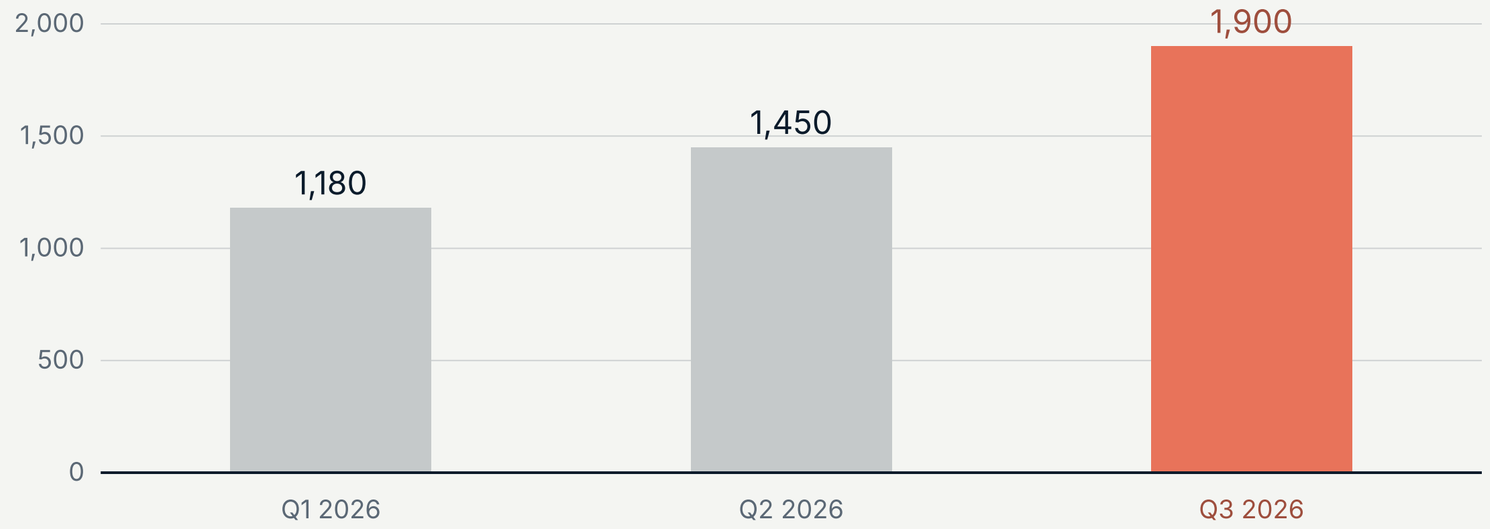
£9.5M

Series A round

TRACTION

Bookings reached £1.9M in Q3 2026, up 31% on Q2

GMV, £ THOUSAND PER QUARTER, FINANCE FIGURES



SOURCE: DOCS/BOARD-UPDATE-2026-Q3.MD

31%

growth, Q2 to Q3 2026; the Q2 base is being reconciled

£7.6M

GMV run-rate, Q3 2026 annualised

Finance is the system of record. The weekly booking export shows £1.73M for Q3 because it leaves out manually invoiced enterprise bookings.

THE PROBLEM

An employee in pain waits about 14 weeks for NHS physiotherapy, by our estimate

The wait
About 14 weeks on the NHS in most areas; 10 days or more through an insurer's pathway (our estimates)

Who pays
The employer carries the absence either way, most of all in logistics, care, manufacturing and retail

Meanwhile
Independent clinics have empty slots on weekday mornings and afternoons

SOURCE: NOTES/FOUNDER-PERSPECTIVE.MD, DOCS/COMPETITIVE-LANDSCAPE.MD

14 weeks

NHS physiotherapy wait, our estimate; public source to be added

19 days

to a first appointment through one employer's previous occupational health route, January to June 2025, n=212

DOCS/CLINICAL-OUTCOMES-WHITEPAPER.MD

BEFORE: OCCUPATIONAL HEALTH ROUTE

~~19~~ days

Average wait for a first appointment, January to June 2025

WITH KESTREL

2.3 days

The same 212 employees, January to June 2026

THE SOLUTION

With Kestrel, the first appointment came in 2.3 days, not 19

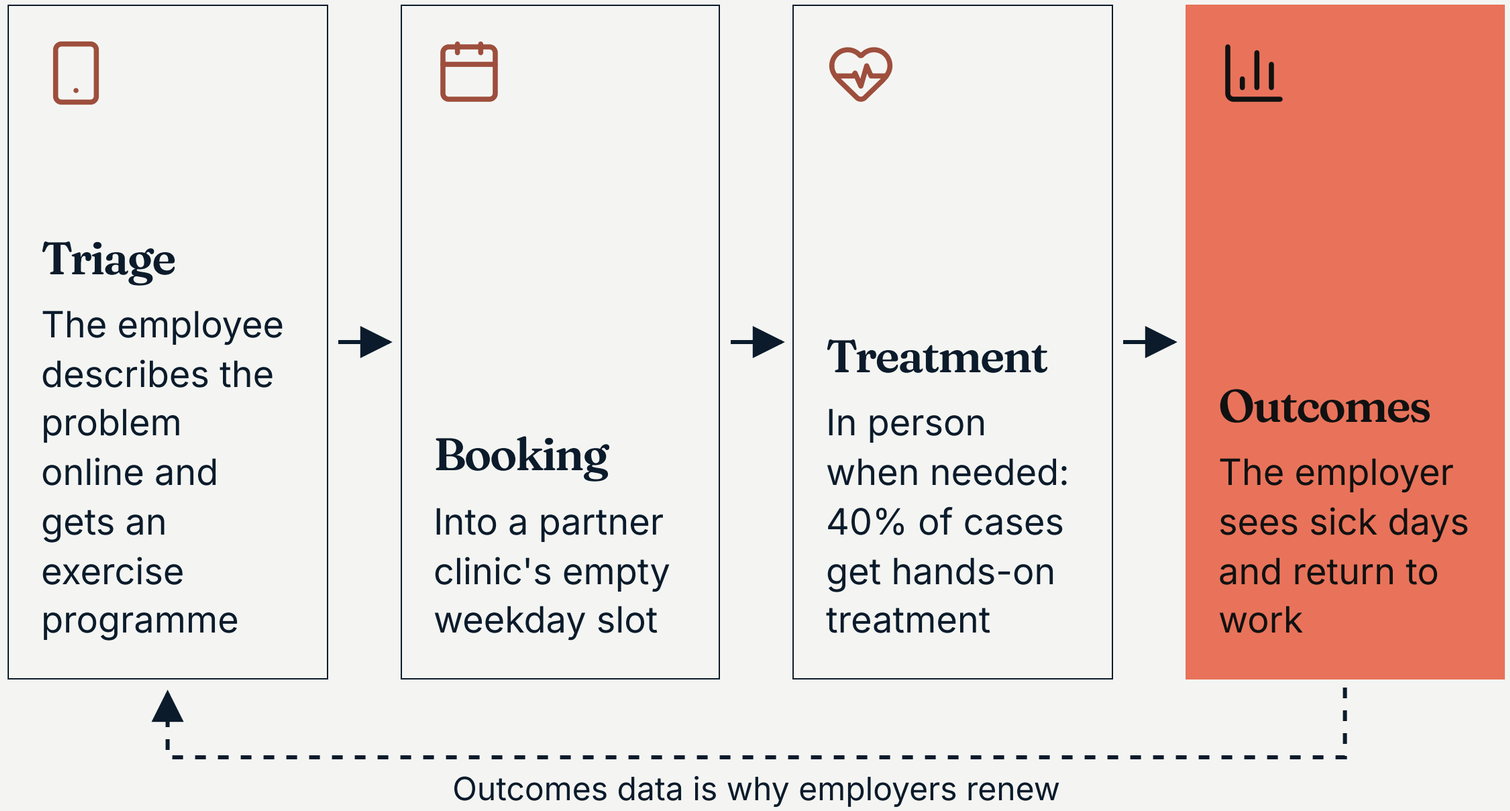
One logistics employer, 212 employees, January to June 2026

- Digital triage first, then a physiotherapist in person when the case needs hands
- 312 partner clinics, booked into slots that would otherwise sit empty
- The employer sees sick days and return to work for its own people

SOURCE: DOCS/CLINICAL-OUTCOMES-WHITEPAPER.MD, DATA/KPIS-WEEKLY.CSV

PRODUCT

One booking connects the employee, a clinic's empty slot and the employer's absence data



- Live today: booking engine, clinic app and employer dashboard, sold on annual contracts
- 30,100 bookings in Q3 2026 across 312 active clinics
- Planned with this round: outcomes tracking, clinician tooling, HR system integrations

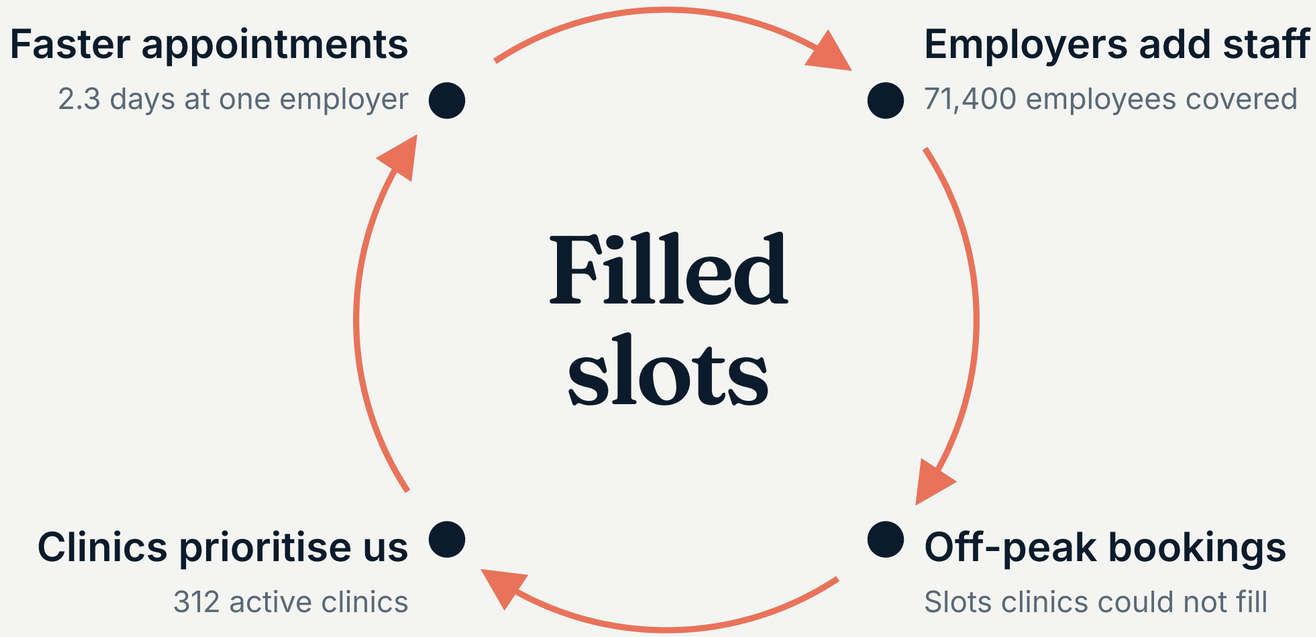
SOURCE: NOTES/FOUNDER-PERSPECTIVE.MD, DOCS/BOARD-UPDATE-2026-Q3.MD, DATA/KPIS-WEEKLY.CSV

INSIGHT

Clinics take a 22% cut gladly: the alternative is an empty 11am Tuesday

22%

of each booking kept by Kestrel, Q3 2026



SECTION 01

01

Proof: employers stay, and spend more

Network growth, one employer's outcomes, unit economics and retention

TRACTION

41 employers and 312 clinics were active at the end of September 2026

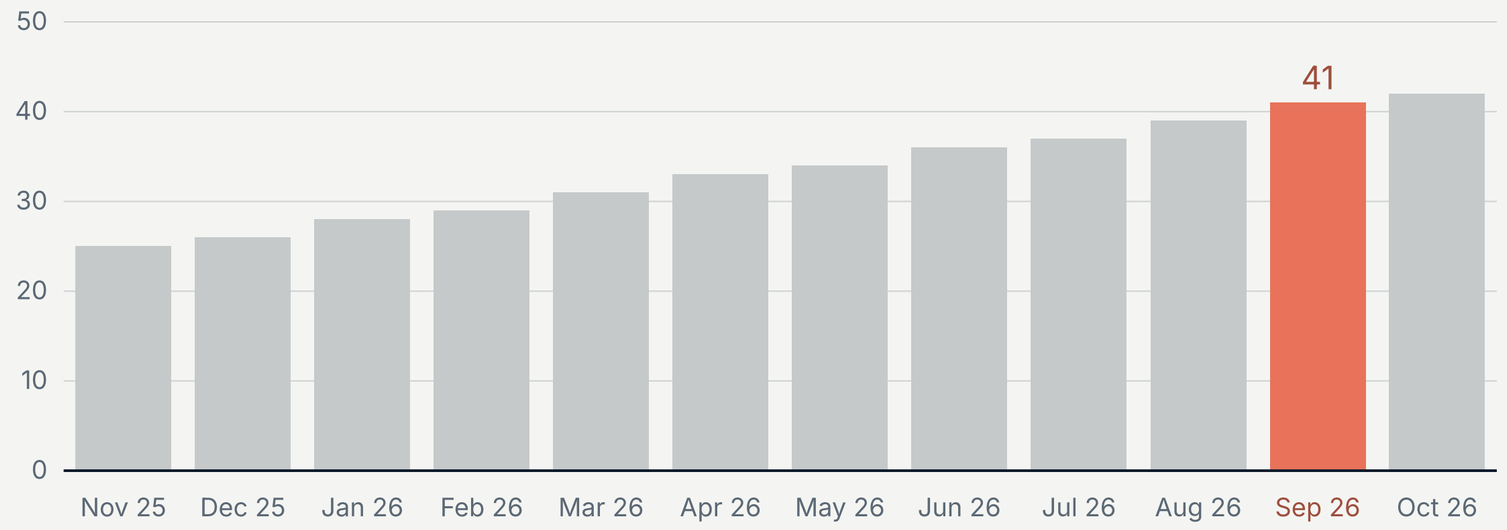
41

active employers, end of September 2026

312

active partner clinics, end of September 2026

ACTIVE EMPLOYERS, LAST WEEK OF EACH MONTH, WEEKLY EXPORT

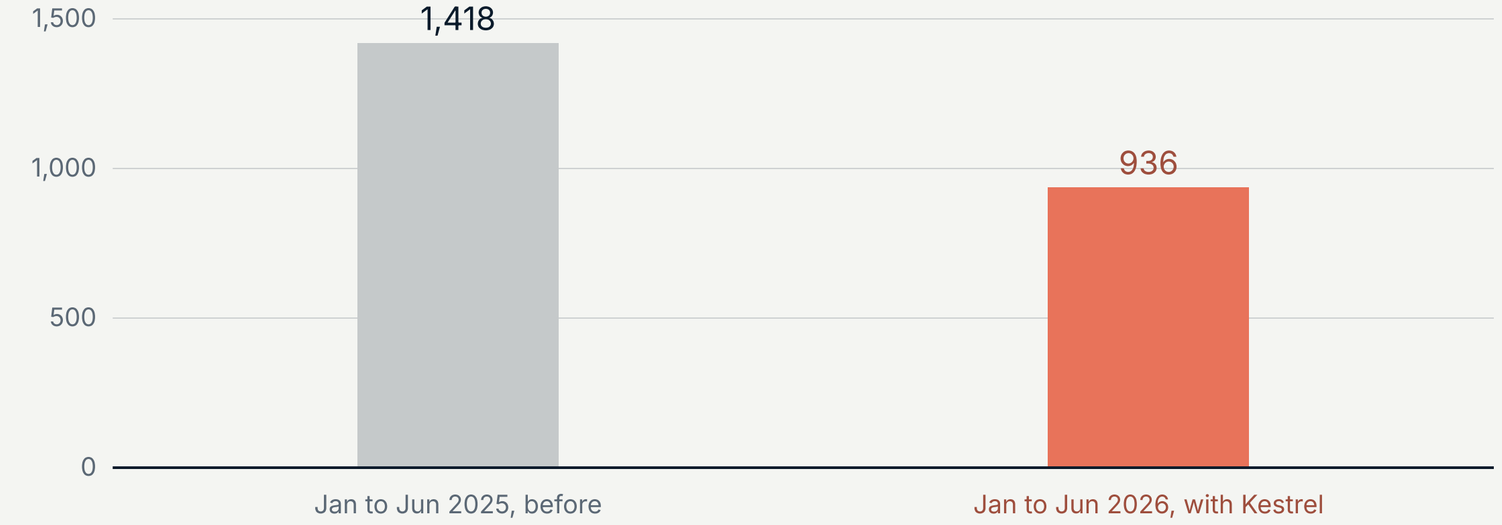


SOURCE: DATA/KPIS-WEEKLY.CSV

Counts come from the weekly export. The board update's March and June counts are two lower; the two are being reconciled.

CASE STUDY

At one logistics employer, MSK sick days fell 34% among 212 employees



SOURCE: DOCS/CLINICAL-OUTCOMES-WHITEPAPER.MD

34%

fewer MSK sick days, n=212, no control group

2.3 days

to a first appointment, down from 19

- One employer, two sites; the same employees a year apart
- No control group; employees chose the pathway; shifts changed in March 2026
- Controlled evaluation across 4 employers planned for 2027

Insurers optimise for claims cost. Employers optimise for people being at work.

Those are different products. Kestrel sells to the employer.

BUSINESS MODEL

Kestrel keeps 22% of each booking, at a 61% gross margin

22%

take rate: Kestrel's share of each booking, Q3 2026

61%

gross margin on net revenue, Q3 2026

118%

net revenue retention, all employers, 12 months to September 2026

9 months

CAC payback, blended, median employer; our target was 6



Employers pay for each booking through Kestrel and the clinic receives the rest: £418k of net revenue on £1.9M of bookings in Q3 2026



Enterprise employers repay their acquisition cost in 1.9 months (n=14); the table is in the appendix

INSIGHT

Employers who own absence as a metric stay: none of 14 enterprise employers has left

0 of 14

enterprise employers lost in two years
(small sample, short record)

Where we lost

7 employers left in 12 months, every one with fewer than 200 employees

Why

Too few employees book, so finance cancels at renewal

What we changed

From Q4 2026 we sell only to employers with 250 or more staff

SECTION 02

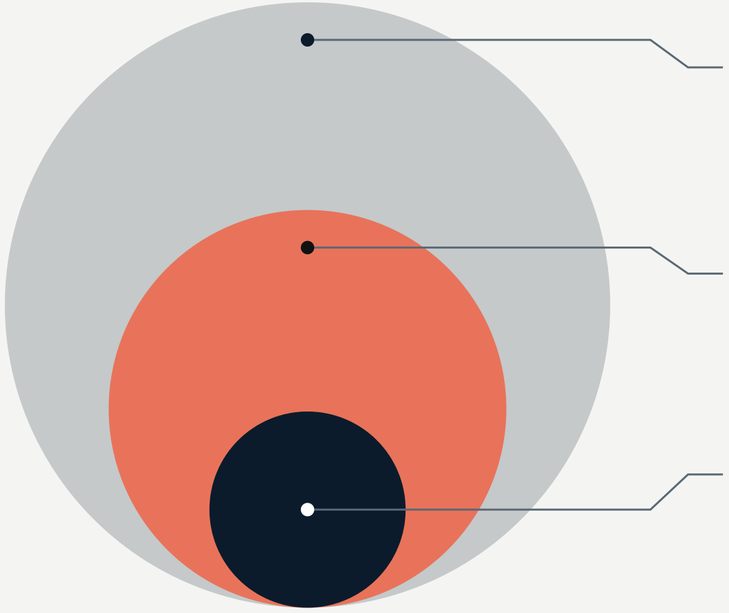
02

The market, and why Kestrel wins it

The serviceable market, how we sell, and the alternatives an employer has

MARKET

We estimate £1.1B of serviceable bookings a year: 15.6M employees x 18% x £403



£1.1B

Serviceable bookings: 15.6M employees x 18% x £403 (assumption)

£242M

Net revenue pool: serviceable bookings x our 22% take rate (assumption)

£1.7M

Kestrel today: net revenue run-rate, Q3 2026 annualised (fact)

- Employees at 250+ employers in the UK, Netherlands and Ireland. 18% report an MSK problem in a year; fewer seek treatment, so the real figure is lower.
- £403 is the market average spend per case, at 6.2 sessions. Booking value, not Kestrel revenue. Public sources for the three inputs are being added.

SOURCE: DOCS/MARKET-SIZING-2026.MD, DATA/UNIT-ECONOMICS.XLSX

GO TO MARKET

Enterprise sales wins a 250+ employer for £11,400, repaid in 1.9 months

£11,400

CAC per enterprise employer, 12 months to September 2026

5 months

average enterprise sales cycle, Q3 2026



Beachhead

UK employers with 250 or more staff in logistics, care, manufacturing and retail, where someone owns absence



Motion

An enterprise sales team led by a VP Sales with 10 years in employee benefits; the outcomes whitepaper opens the conversation



Next

First market outside the UK with this round: the Netherlands or Ireland

COMPETITION

Only Kestrel pairs in-person treatment in days with absence data for the employer

	Kestrel Health	INSURER MSK PATHWAYS	US DIGITAL MSK PROVIDERS	NHS
Wait for a first appointment	**2.3 days (one employer, n=212)**	10+ days	no clinic visit	about 14 weeks
Hands-on treatment in a clinic	●	●	○	●
Outcomes data for the employer	●	○	●	○
Already in the employer's benefits	○	●	○	free to all
Brand and scale of evidence	**one employer so far**	established	ahead of us	n/a

- Risks we name: our clinic network is not exclusive, and an insurer has talked about 48-hour access.

SOURCE: DOCS/COMPETITIVE-LANDSCAPE.MD, DOCS/CLINICAL-OUTCOMES-WHITEPAPER.MD, NOTES/FOUNDER-PERSPECTIVE.MD

SECTION 03

03

The team and the round

Who builds Kestrel, what £9.5M buys, and what will be true at the end of 2027

TEAM

We sold health benefits to employers, ran physiotherapy clinics and built payments infrastructure

PR

Priya Raghunathan

CEO AND CO-FOUNDER

6 years at a UK health insurer selling benefits to large employers; owned a £120M book

CF

Callum Fairbairn

COO AND CO-FOUNDER

Chartered physiotherapist, 12 years clinical; ran and sold a 4-site physiotherapy group

MK

Mateusz Kowal

CTO AND CO-FOUNDER

Staff engineer in payments at a UK fintech; built the booking engine, clinic app and employer dashboard

GN

Grace Ndlovu

VP SALES, ENTERPRISE

10 years selling employee benefits at two insurers and a benefits platform; joined September 2025

- 24 people: 9 in engineering and product, 6 in sales and employer success, 5 in clinical and clinic partnerships

SOURCE: TEAM/TEAM.MD

THE ASK

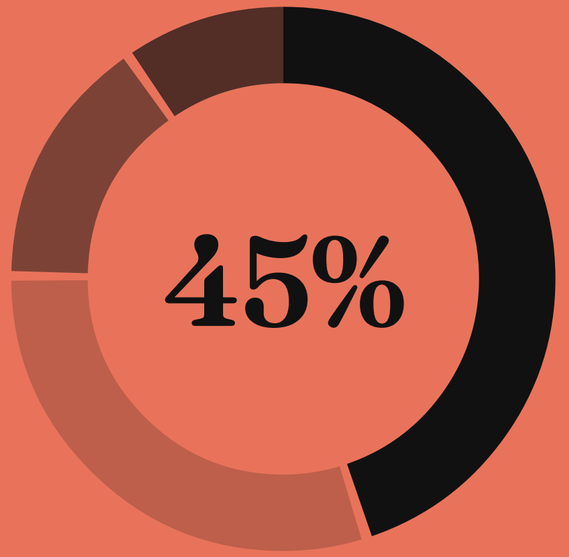
Raising £9.5M to reach a £10M GMV run-rate and 120 employers by end of 2027

Runway after the round: needs number months

WHAT IT BUYS

- **End of 2027** £10M GMV run-rate (projection; £7.6M today)
- **End of 2027** 120 active employers (projection; 41 today)
- **Date to be set** First market outside the UK live: the Netherlands or Ireland

USE OF FUNDS



- Go-to-market: enterprise sales and employer success **45%**
- Product: outcomes tracking, clinician tooling, HR system integrations **30%**
- Clinical team and supply operations **15%**
- Buffer **10%**

SOURCE: DOCS/BOARD-UPDATE-2026-Q3.MD, DATA/KPIS-WEEKLY.CSV

Fast physiotherapy for employees, in clinics' empty weekday slots

PRIYA RAGHUNATHAN, CEO. CONTACT: needs detail

£1.9M

of bookings in Q3 2026, up 31% on Q2

118%

net revenue retention; none of 14 enterprise employers has left

£9.5M

to reach a £10M GMV run-rate and 120 employers by end of 2027

APPENDIX

Unit economics: enterprise employers repay acquisition cost in 1.9 months

12 MONTHS TO SEPTEMBER 2026	MEDIAN EMPLOYER, BLENDED	ENTERPRISE EMPLOYER (N=14)
Acquisition cost per employer	£6,800	£11,400
Monthly net revenue per employer	£1,241 (median)	£9,900 (mean)
Gross margin on net revenue	61%	61%
CAC payback	9 months (target 6)	1.9 months

- Acquisition cost is fully loaded sales and marketing spend over new employers won. Payback divides it by monthly net revenue after gross margin.

SOURCE: DATA/UNIT-ECONOMICS.XLSX

APPENDIX

The risks we name ourselves, and what we are doing about each

58%

of GMV from the five largest employers, Q3 2026; none of 14 enterprise employers has left in two years

8.8 months

of cash at Q3 net burn, end of September 2026



Evidence

One employer, no control group; controlled evaluation planned for 2027



Supply

Clinics are not exclusive; we fill slots they could not fill otherwise



Data

Finance and the booking export differ by quarter; being reconciled

APPENDIX

Sources

48 figures from 8 source files. Each figure has an entry in the fact ledger (facts.yaml) that names its file and location.

SOURCE FILE	FIGURES	KINDS	SLIDES
docs/board-update-2026-Q3.md	18	12 facts, 2 projections, 4 assumptions	1, 2, 5, 6, 11, 14, 15, 19, 22
data/kpis-weekly.csv	4	4 facts	1, 2, 4 to 6, 8, 19
notes/founder-perspective.md	3	1 assumption, 2 facts	3, 5, 12, 16
docs/competitive-landscape.md	2	2 assumptions	3, 16
docs/clinical-outcomes-whitepaper.md	4	4 facts	3, 4, 6, 9, 16
data/unit-economics.xlsx	12	11 facts, 1 assumption	6, 11, 12, 14, 15, 21, 22
docs/market-sizing-2026.md	5	5 assumptions	14
team/team.md	1	1 fact	18